Quiz 1 Practice Questions — Chapters 1-8

Chapter 1: Introduction to Ethics

1. The term "moral" is derived from the Latin word *mores*, meaning customs or norms. Moral actions are determined within a broad cultural context or religious standard.

- **True** or **False**?

2. Which of the following best describes the relationship between law, ethics, and best practice?

- A) Ethics represents the minimum standard; law represents the ideal
- B) Law represents the minimum standard society will tolerate; ethics occupies a middle ground; best practice represents the ideal
- C) Best practice is enforceable; ethics and law are aspirational
- D) All three are identical in scope and enforceability

3. A counselor who functions at a level that merely keeps them in compliance with minimal ethical standards is practicing:

- A) Aspirational ethics
- B) Virtue ethics
- C) Mandatory ethics
- D) Relational ethics

4. Which ethical theory evaluates an action based on the *principle* that guides it, regardless of consequences?

- A) Utilitarianism
- B) Deontology
- C) Ethical relativism
- D) Egoism

5. The ACA Code of Ethics was first adopted in:

- A) 1974
- B) 1953
- C) 1961
- D) 2005

6. In the model for professional practice (Figure 1-1), which element forms the BASE of the internal building blocks?

- A) Self-awareness
- B) Moral principles and virtues
- C) Intentionality
- D) Knowledge of ethical, legal, and professional standards

7. According to the textbook, the ethical perspective focuses more on protecting the _____, whereas the legal perspective focuses more on protecting the _____.

- A) counselor; client
- B) client; counselor
- C) profession; public
- D) public; profession

8. Ethical relativists believe that:

- A) Absolute moral standards exist that must apply in all circumstances
- B) Moral standards do not exist at all
- C) If members of a culture believe an action is morally right, then it is morally right within that culture
- D) Consequences are the only thing that matters in ethical decision-making

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Chapter 2: Professional Identity of Counselors

9. The consensus definition of counseling agreed upon by 29 organizations states: "Counseling is a professional relationship that empowers diverse individuals, families, and groups to accomplish _____."

- A) mental health and career goals
- B) mental health, wellness, education, and career goals
- C) psychological wellness and personal growth
- D) recovery from mental illness

10. The terminal (required) degree for licensure as a professional counselor is:

- A) A doctoral degree (PhD or EdD)
- B) A bachelor's degree
- C) A master's degree
- D) An educational specialist degree

11. Which of the following is NOT one of the four foundational beliefs of the counseling profession?

- A) The wellness model of mental health
- B) A developmental perspective
- C) The medical model of diagnosis and treatment
- D) Empowerment of clients

12. CACREP stands for:

- A) Council for the Advancement of Counseling and Related Educational Programs
- B) Council for Accreditation of Counseling and Related Educational Programs
- C) Commission for Accreditation of Counselor Education and Related Programs
- D) Council for Accreditation of Clinical and Rehabilitation Education Programs

13. The difference between *licensure* and *national voluntary certification* (such as NCC) is:

- A) Licensure is voluntary; certification is required by law
- B) Licensure is established by state law and is typically required for practice; national certification (e.g., NCC through NBCC) is voluntary
- C) There is no meaningful difference between the two
- D) National certification is more powerful than state licensure

14. The first counselor licensure law in the United States was passed in:

- A) New York in 1952
- B) California in 1985
- C) Virginia in 1976
- D) Ohio in 2018

15. A key difference between counselors and psychiatrists is that:

- A) Psychiatrists use the wellness model; counselors use the medical model
- B) Counselors require a doctoral degree; psychiatrists require a master's degree
- C) Psychiatrists are physicians trained in the medical model with prescribing ability; counselors adopt the wellness model with a master's degree
- D) There are no meaningful differences in training or approach

16. The counseling profession's preference for *prevention and early intervention* rather than remediation is one of its four foundational beliefs.

- **True** or **False**?

Chapter 3: Multiculturalism, Social Justice, Advocacy, and Values

17. According to ACA Code of Ethics Standard A.4.b., counselors are required to:

- A) Share their personal values with clients to build rapport
- B) Be aware of and avoid imposing their own values, attitudes, beliefs, and behaviors on clients
- C) Only work with clients who share their values
- D) Refer any client whose values differ from their own

18. *Bracketing* in counseling refers to:

- A) Setting time limits on counseling sessions
- B) The intentional separating of a counselor's personal values from their professional values to provide ethical counseling to all clients
- C) A technique for organizing case notes
- D) Limiting the topics discussed in session

19. According to ACA Code A.11.b., it is appropriate for a counselor to refer a client based solely on the counselor's personally held values.

- **True** or **False**?

20. The ADDRESSING model (Hays, 2002) is used to:

- A) Write treatment plans
- B) Consider multiple dimensions of a client's cultural identity in assessment and treatment
- C) Diagnose mental disorders
- D) Organize case notes

21. *Intersectionality* (Crenshaw, 1989) refers to:

- A) The intersection of counseling theory and practice
- B) The compounded oppression experienced by individuals with multiple minority identities
- C) The overlap between legal and ethical standards
- D) Where different ACA Code sections address the same issue

22. When is it appropriate for a counselor to refer a client according to ACA standards?

- A) When the counselor disagrees with the client's lifestyle choices
- B) When the counselor lacks competence in the clinical area needed to assist the client
- C) When the counselor finds the client's values offensive
- D) When the counselor and client are of different races

23. Multicultural counseling competency is required in:

- A) Only school counseling settings

- B) Only clinical mental health settings
- C) All counseling specialties (ACA C.2.a.)
- D) Only when working with immigrant populations

24. Cultural humility includes a willingness to seek consultation, seek continuing education, and make referrals — and recognizes that the most important resource for learning about cultural differences is the CLIENT.

- **True** or **False**?

Chapter 4: Client Welfare and Informed Consent

25. A counselor's *fiduciary relationship* with a client means:

- A) The counselor manages the client's finances
- B) The counselor acts in the client's benefit; the client is not there for the counselor's benefit
- C) The client must pay the counselor before receiving services
- D) The counselor and client share equal responsibility for outcomes

26. Written contracts for informed consent are commonly referred to as:

- A) Treatment plans
- B) Disclosure statements
- C) Progress notes
- D) Liability waivers

27. Which of the following is NOT required in an informed consent document?

- A) Confidentiality and its limitations
- B) The counselor's personal religious beliefs
- C) Emergency protocols and crisis resources
- D) Fees and billing information

28. Informed consent is a one-time event that only needs to happen at the first session.

- **True** or **False**?

29. A *countertransference reaction* occurs when:

- A) The client projects past feelings onto the therapist
- B) The counselor's own feelings, attitudes, and experiences are triggered by the client
- C) The counselor refers the client to another provider
- D) The client refuses treatment

30. Which of the following could lead to legal problems if not handled properly in informed consent?

- A) Guaranteeing an outcome as a result of counseling
- B) Guaranteeing privacy with no exceptions
- C) Misrepresenting credentials
- D) All of the above

31. When working with mandated clients, counselors must:

- A) Refuse to see them since they did not seek counseling voluntarily
- B) Explain the consequences of refusal, be accurate and honest in evaluations, and clarify limits of privacy
- C) Treat them exactly the same as voluntary clients with no modifications

- D) Disclose everything to the court without informing the client

32. Treatment plans and goals should be established:

- A) By the counselor alone based on their expertise
- B) By the client's insurance company
- C) Jointly with the client
- D) By the counselor's supervisor

Chapter 5: Confidentiality and Privileged Communication

33. What is the difference between *confidentiality* and *privileged communication*?

- A) They are the same concept
- B) Confidentiality is an ethical obligation to not reveal client information; privileged communication is a legal protection against forced disclosure in court
- C) Privileged communication applies to all conversations; confidentiality only applies in court
- D) Confidentiality is a legal concept; privileged communication is an ethical concept

34. Confidentiality is the only specific ethical issue to which an entire section of the ACA Code of Ethics is devoted (Section B).

- **True** or **False**?

35. The landmark case *Tarasoff v. University of California* (1976) established:

- A) The right to privileged communication
- B) The duty to warn and protect when a client poses a danger to an identifiable third party
- C) That counselors can never break confidentiality
- D) That minors cannot consent to treatment

36. In Washington State, the *Volk v. DeMeerleer* case extended the duty to protect by ruling that:

- A) Therapists only have a duty when a client names a specific victim
- B) A therapist's duty is NOT limited to a duty to warn — the duty to protect can arise even if the client does NOT express an intention to harm a reasonably identifiable victim
- C) Therapists have no duty to protect third parties
- D) Only psychiatrists have a duty to protect

37. Privileged communication belongs to:

- A) The counselor
- B) The client
- C) The court
- D) The counselor's employer

38. In Washington State, minors age 13 and older may request and receive outpatient mental health treatment without parental consent.

- **True** or **False**?

39. Which of the following is an exception to confidentiality?

- A) Client waiver/release of information
- B) Imminent danger to self or others
- C) Suspected child abuse or neglect

- D) All of the above

40. The *Jaffee* case established that:

- A) Counselors must always break confidentiality when subpoenaed
- B) Communications between licensed psychotherapists and their clients are privileged and protected from forced disclosure in federal cases
- C) Privileged communication does not apply to counselors
- D) All client records must be turned over to insurance companies

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Chapter 6: Records and Subpoenas

41. According to ACA Code B.6.a., counselors must:

- A) Keep minimal records to protect client privacy
- B) Maintain documentation necessary for rendering professional services
- C) Only keep records if required by their employer
- D) Allow anyone to access client records upon request

42. SOAP notes stand for:

- A) Summary, Outline, Assessment, Prognosis
- B) Subjective, Objective, Assessment, Plan
- C) Standardized, Organized, Accurate, Professional
- D) Session, Observation, Analysis, Prescription

43. In Washington State, records must be kept for a minimum of _____ years after termination (per WAC 246-809-035).

- A) 3 years
- B) 5 years
- C) 7 years
- D) 10 years

44. When a counselor is subpoenaed, the first thing they should do is immediately hand over all client records.

- **True** or **False**?

45. HIPAA requires counselors to:

- A) Give clients a clear written explanation of how they use, keep, and disclose health care information
- B) Give clients access to their records (unless it would cause them harm)
- C) Follow a written process for clients to request amendments to their records
- D) All of the above

46. Counselors should write clinical notes with the understanding that:

- A) No one will ever see them
- B) Someone may read them at a later date, including the client or a court
- C) They are only for the counselor's personal use
- D) They should contain the counselor's personal opinions about the client

47. If a client requests their records, a counselor should:

- A) Refuse the request entirely

- B) Provide the records immediately without any process
- C) Get a signed release, explain risks of disclosure, ensure records would not harm the client, and provide assistance interpreting the records
- D) Only provide records if a lawyer requests them

48. Subpoenas can be used for all of the following EXCEPT:

- A) Obtaining copies of records
- B) Requiring witnesses to testify at a deposition
- C) Automatically overriding privileged communication
- D) Requiring witnesses to appear and bring records

Chapter 7: Competence, Assessment, and Diagnosis

49. The most salient moral principle related to counselor competency is:

- A) Autonomy
- B) Justice
- C) Nonmaleficence (do no harm)
- D) Veracity

50. Licensure is best described as:

- A) A voluntary national credential
- B) The most powerful type of credentialing, established by state law, typically required for practice
- C) A certification that shows specialty knowledge
- D) A recommendation from a former professor

51. Which of the following is NOT a way to maintain counselor competence?

- A) Continuing education
- B) Peer consultation
- C) Avoiding difficult cases entirely
- D) Making appropriate referrals

52. Burnout in counselors can lead to:

- A) Decline in self-care
- B) Strain in personal relationships
- C) Damage to overall emotional health
- D) All of the above

53. According to ACA Code C.2.g., if counselors determine they are impaired, they must:

- A) Continue practicing while seeking help
- B) Limit, suspend, or terminate their professional responsibilities and seek assistance
- C) Ignore the impairment and hope it resolves
- D) Only inform their supervisor but continue seeing clients

54. When using the DSM-5-TR for diagnosis, counselors should:

- A) Avoid informing clients about their diagnosis
- B) Ensure clients understand the implications of any diagnosis assigned to them
- C) Assign the least stigmatizing diagnosis regardless of accuracy
- D) Only use the DSM if required by insurance

55. Referrals should be based on:

- A) The counselor's personal discomfort with the client
- B) Competence (the counselor lacks skills in the needed area), NOT discriminatory reasons
- C) The client's race, religion, or sexual orientation
- D) Whether the counselor likes the client

56. Risk factors for counselor impairment include environmental factors, vicarious traumatization, and preexisting conditions.

- **True** or **False**?

Chapter 8: Malpractice and Resolving Legal and Ethical Challenges

57. To prevail in a malpractice lawsuit, a plaintiff must prove all of the following EXCEPT:

- A) The counselor had a duty to the client
- B) The counselor failed to conform to the required duty of care
- C) The counselor intentionally tried to harm the client
- D) The client was injured and there was a causal connection between the counselor's conduct and the injury

58. The most common reason mental health professionals are sued for malpractice is:

- A) Incorrect diagnosis
- B) Sexual relationships with clients
- C) Breaking confidentiality
- D) Overcharging for services

59. According to Washington State law (WAC 246-16-100), a counselor may never engage in a sexual relationship with a current client or a FORMER client. This is MORE restrictive than the ACA Code of Ethics.

- **True** or **False**?

60. When assessing suicide risk, counselors should:

- A) Only ask indirect questions to avoid upsetting the client
- B) Take direct verbal warnings seriously, assess for a plan, means, and prior attempts
- C) Assume that talking about suicide will cause the client to attempt it
- D) Wait for the client to bring up suicidal thoughts before assessing

61. The legal standard of care for counselors is:

- A) Perfect outcomes for every client
- B) Practice consistent with how a reasonable, similarly educated counselor would practice under the same circumstances
- C) Following every ethical code to the letter with no deviation
- D) Whatever the counselor's supervisor says to do

62. If a formal ethics complaint is filed against you, you should:

- A) Contact the client who filed the complaint to work things out
- B) Ignore it and hope it goes away
- C) Respond fully in writing, consult with an attorney, submit documentation, and contact your liability insurance carrier
- D) Immediately resign from your position

63. *Proximate cause* in malpractice refers to:

- A) The closest hospital to the counselor's office
- B) A reasonably close causal connection between the counselor's conduct and the client's injury
- C) The approximate cost of legal proceedings
- D) The counselor's proximity to the client during sessions

64. Non-suicidal self-injury (NSSI) and suicidal ideation are the same behavior and should be assessed the same way.

- **True** or **False**?

ANSWER KEY

Chapter 1

1. **True** (Ch. 1, p. 3 — derived from Latin *mores*)
2. **B** (Class 1 slides p. 47; Ch. 1, pp. 5-6)
3. **C** (Ch. 1, p. 15)
4. **B** (Ch. 1, p. 10 — Kant's deontology)
5. **C** (Ch. 1, p. 15 — initiated 1953, adopted 1961)
6. **C** (Ch. 1, pp. 7-8 — intentionality is the base)
7. **B** (Ch. 1, p. 10)
8. **C** (Ch. 1, p. 10)

Chapter 2

9. **B** (Class 1 slides p. 25; Ch. 2, p. 29)
10. **C** (Class 1 slides p. 35; Ch. 2, p. 35)
11. **C** (Class 1 slides pp. 29-32; Ch. 2, pp. 30-34 — counselors use the wellness model, not the medical model)
12. **B** (Ch. 2, p. 36)
13. **B** (Class 4 slides pp. 24-26; Ch. 2, pp. 36-39)
14. **C** (Ch. 2, p. 45 — Virginia, 1976)
15. **C** (Class 1 slides p. 28; Ch. 2, pp. 56-59)
16. **True** (Class 1 slides pp. 31; Ch. 2, pp. 33-34)

Chapter 3

17. **B** (Class 2 slides p. 36; ACA A.4.b.)
18. **B** (Class 2 slides p. 35; Kocet & Herlihy, 2014)
19. **False** (ACA A.11.b. — counselors refrain from referring based solely on personally held values)
20. **B** (Class 2 slides p. 15)
21. **B** (Class 2 slides p. 15; Crenshaw, 1989)
22. **B** (Class 2 slides pp. 38-39; ACA A.11.a.)
23. **C** (Class 2 slides p. 10; ACA C.2.a.)
24. **True** (Class 2 slides p. 13)

Chapter 4

25. **B** (Class 2 slides p. 57; Class 3 slides p. 7)
26. **B** (Class 2 slides p. 66; Class 3 slides p. 21; Ch. 4, p. 99)

27. **B** (Class 2 slides p. 70; Class 3 slides p. 26)
28. **False** (Class 3 slides p. 23 — informed consent is an ongoing process, both oral and written)
29. **B** (Class 2 slides p. 58; Class 3 slides p. 11)
30. **D** (Class 2 slides p. 71; Class 3 slides p. 25)
31. **B** (Class 2 slides p. 59; Class 3 slides p. 14; ACA A.2.e., C.6.b.)
32. **C** (Class 2 slides p. 60; Class 3 slides p. 15)

Chapter 5

33. **B** (Class 3 slides p. 37; Ch. 5, pp. 110-115)
34. **True** (Class 3 slides p. 41; Ch. 5)
35. **B** (Class 3 slides p. 49)
36. **B** (Class 3 slides pp. 50-51)
37. **B** (Class 3 slides pp. 69, 71; Ch. 5, p. 119)
38. **True** (Class 3 slides p. 43; RCW 71.34.530)
39. **D** (Class 3 slides p. 46; Ch. 5, pp. 121-130)
40. **B** (Class 3 slides p. 52)

Chapter 6

41. **B** (Class 4 slides p. 4; ACA B.6.a.)
42. **B** (Class 4 slides pp. 6-7)
43. **B** (Class 4 slides p. 4; WAC 246-809-035)
44. **False** (Class 4 slides p. 18 — get advice before responding, do not just hand over records)
45. **D** (Class 4 slides p. 15)
46. **B** (Class 4 slides p. 5)
47. **C** (Class 4 slides p. 12)
48. **C** (Subpoenas do not automatically override privileged communication; Class 4 slides pp. 17-18)

Chapter 7

49. **C** (Class 4 slides p. 21; Ch. 7, p. 161)
50. **B** (Class 4 slides p. 24; Ch. 7, pp. 164-165)
51. **C** (Class 4 slides pp. 28-31; Ch. 7, pp. 167-172)
52. **D** (Class 4 slides p. 32; Ch. 7, pp. 173-176)
53. **B** (Class 4 slides p. 36; ACA C.2.g.)
54. **B** (Class 4 slides pp. 46-48; Ch. 7, pp. 187-193)
55. **B** (Class 4 slides p. 21; ACA A.11.a.)
56. **True** (Class 4 slides p. 33; Ch. 7, p. 174)

Chapter 8

57. **C** (Class 4 slides p. 52; Ch. 8, p. 197 — malpractice does NOT require proving intentional harm, only failure to meet duty of care)
58. **B** (Class 4 slides p. 53; Ch. 8, p. 197)
59. **True** (Class 4 slides p. 53; WAC 246-16-100 — WA law is more restrictive than ACA, which has a 5-year waiting period)
60. **B** (Class 4 slides pp. 61-62; Ch. 8, pp. 199-207)
61. **B** (Ch. 8; Ch. 1, p. 26)
62. **C** (Class 4 slides p. 54; Ch. 8, pp. 215-219)
63. **B** (Class 4 slides p. 52; Ch. 8, p. 197)

64. **False** (Class 4 slides p. 60; Ch. 8 — NSSI and suicidal ideation are two separate behaviors and should be assessed separately)